



Non-Invasive Prenatal Test

Requisition and Consent Form

Barcode

*All required fields MUST be filled in.

Patient Info.	Name*				MRN	
	Date of Birth*	DD / MM / YYYY	City/State		Country	
	Ethnicity	☐ East Asian ☐ Southeast Asian ☐ African ☐ Caucasian ☐ Hispanic ☐ Other				
Obstetrical History	☐ Pregnancy with genetic disorder : ☐ Blood transfusion/Stem cell treatment : ☐ Other specifications :				Abortion history	☐ Yes (times) ☐ No
Clinical Info.	No. of fetus*	☐ Singleton ☐ Twin	Gestational age* (by ultrasound)	weeks days	IVF application	☐ Yes ☐ No
	Patient* height/weight	cm	kg	BMI:	Prenatal biochemical screening test	☐ Yes (high risk) ☐ Yes (low risk) ☐ No
	Significant features	(ex. Ultrasonography, vanishing twin, etc)			NT	mm
Specimen	Date of collection*	DD / MM / YYYY		Hr : Min	☐ AM ☐ PM	

Test Selections

*SCA : Sex chromosomal aneuploidies

Test Request	☐ G-NIPT Lite (T21, T18, T13) ☐ G-NIPT Basic (T21, T18, T13 + SCA) ☐ G-NIPT Premium (T21, T18, T13 + SCA* + Other Chromosomes + Microdeletions)	☐ Fetal Sex (Optional)
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G-NIPT Test Features

- G-NIPT is a noninvasive prenatal screening test for detecting numerical chromosome abnormalities such as T21/T18/T13(Down/Edward/Patau syndrome).
- G-NIPT Basic screens T21/T18/T13 and sex chromosomal aneuploidies [XO/XXY/XXX/XXY(Turner/Klinefelter/Triples-X syndrome/Jacob's syndrome)].
- G-NIPT Premium screens T21/T18/T13, sex chromosomal aneuploidies [XO/XXY/XXX/XXY(Turner/Klinefelter/Triples-X syndrome/Jacob's syndrome)], deletion syndromes [1p36, 2q33.1, 5p15 (Cri-du-chat), 11qter (Jacobsen)] and other chromosomes.
- G-NIPT does not report fetal sex, sex chromosome abnormalities and microdeletion syndromes in the case of a twin fetus.

G-NIPT Test Limitations

- G-NIPT is highly sensitive but not a confirmatory test. It is recommended that a high risk result and/or other clinical indications of a chromosomal abnormality be confirmed through fetal karyotype analysis such as amniocentesis. A low risk result does not guarantee an unaffected pregnancy due to the screening limitations of the test.
- In cases of the patient holding chromosomal aneuploidy, mosaicism, chromosomal microdeletion/duplication, or multiple fetuses, the test result may not be accurate.
- Patient with blood transfusion, stem cell treatment, or transplantation history may receive inaccurate results due to exogenous DNA.
- For a variety of reasons, including biological, the test has a failure rate (insufficient quantity of fetal DNA in maternal blood, or low quality test data due to premature testing, a twin fetus, high BMI, specimen hemolysis, transportation issues, or other unknown factors).
- G-NIPT is not eligible for patients with an excess number of fetuses (more than two fetuses, vanishing triplets).
- G-NIPT may not be eligible for patients with a high BMI (over BMI 27~30). In this case, there is a possibility of no-call result.
- G-NIPT test is performed between 10 weeks and 22 weeks of pregnancy, and in the case of vanishing twins, the test can be performed at least 6 weeks after the disappearance, but it is recommended to test after 9 weeks. Test result done earlier than the recommended time may not be accurate.

Informed Consent for Patient

- I agree to provide accurate personal information.
- I understand the test is not for diagnostic purposes.
- I understand the limitations of the test. Test sensitivity and specificity is high, but 'false negative' or 'false positive' test results still may occur.
- All chromosomal abnormalities of the fetus are analyzed regardless of the test type, but only test options that I have agreed will be reported.
However, I understand that maternal chromosomal abnormalities or other conditions that affect the determination of fetal chromosomal aneuploidy may be reported when discovered.
- I understand that the G-NIPT is not validated for use in the following cases, and therefore the test result may not be accurate.
- singleton: numerical chromosome abnormalities other than T21, T18, T13, sex chromosomes, and chromosomal microdeletion/duplication syndromes.
- twins: numerical chromosome abnormalities, chromosomal microdeletion/duplication syndromes
- I agree that information regarding the sex of the fetus will only be provided under the consent by the patient.
- I agree to provide clinical information after childbirth, particularly when the infant is later affected by chromosomal genetic diseases.
- I agree to my clinical information being used anonymously with all my personal information deleted, for test warranty and research purposes.
- I understand that the test result can be received within 7 days after the specimen arrives at the laboratory. I also understand that the result can be delayed due to natural disasters, emergencies, or any other unavoidable situations.
- I understand redraw may be requested in the cases of low fetal DNA concentration, damage of specimen, or any other unexpected causes. (Test failure rate: 0%~12.2%¹⁾)

Ref. 1) Ultrasound Obstet Gynecol. 2017 Sep;50(3):302-314.

Test Subject (Name : Signature :)

has understood and agreed to all of the ☐ Test Features, ☐ Test Limitations, and ☐ Informed Consent for Patient.

My Counseling Doctor (Hospital : Name : Signature :)

has explained and answered to all of my questions.

Consent Date (DD / MM / YYYY)



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